

Pathogen of the Week

Weekly briefing for policymakers · WHO · Africa CDC · ECDC · US CDC · UKHSA · PHAC · open-source signals

Week 22 · Mon 25 – Sun 31 May 2026

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THIS WEEK AT A GLANCE

The Ebola Bundibugyo outbreak continues to accelerate in DRC and Uganda. As of 24 May, cases exceed 650 suspected with 160+ deaths confirmed and probable. CDC's Title 42 entry restrictions now in effect. The MV Hondius cruise-ship hantavirus cluster remains under international contact-tracing. Mpox clade Ib sustains endemic transmission across EU/EEA with documented vaccination gaps.

Bottom line: The Bundibugyo ebolavirus remains the dominant global public health emergency. As of May 24, DRC and Uganda have reported 650+ suspected cases with 160+ confirmed and probable deaths. The outbreak shows no sign of slowing: 26 new confirmed cases and 143 new suspected cases were identified in the 24–48 hours preceding the 19 May briefing, and a US physician working in Bunia was medevaced to Germany after testing positive. The first American case has galvanized US entry restrictions — CDC invoked Title 42 for the first time since COVID-19 on 18 May, suspending non-US national entry from DRC, Uganda and South Sudan for 30 days. No approved Bundibugyo-specific therapeutics or vaccines exist; Africa CDC and WHO are convening urgent expert review for emergency-use trials of investigational monoclonal antibodies, remdesivir, and pre-clinical vaccine candidates. The MV Hondius Andes hantavirus cluster (10 confirmed/probable, 3 deaths) reached Rotterdam on 18 May; the contact-tracing window spans 19 countries across six weeks. No symptomatic disembarkees were identified on arrival. Mpox clade Ib continues endemic circulation in Europe — 80 clade I cases in 10 EU/EEA countries in March, with only 23% of reported cases fully vaccinated. This week's focus: urgently establish emergency investigational countermeasures (ICM) protocols for Bundibugyo; coordinate international contact-tracing and clinical-alert pathways for hantavirus; sustain targeted vaccination for at-risk MSM populations for mpox.

ACUTE EVENT · LEAD STORY · PHEIC

LEAD

Ebola Bundibugyo

Bundibugyo ebolavirus (BVD) — DRC Ituri outbreak with cross-border spread to Uganda and first American case

DRC Ituri Province (Bunia, Rwampara, Mongbwalu) · Kampala, Uganda · medevac of US doctor to Germany

650+

SUSPECTED CASES (DRC + UGANDA AS OF 24 MAY)

160+

DEATHS (CONFIRMED + PROBABLE)

64

LABORATORY-CONFIRMED CASES

Severity

9.8

Novelty

8.5

Attention

10.0

Policy

10.0

Pandemic

5.0

Composite: 8.7 / 10

WHY IT MATTERS

The PHEIC declared on 16 May has escalated sharply. In nine days (15–24 May), suspected cases surged from ~246 to 650+, deaths nearly doubled from 80 to 160+, and confirmed cases more than doubled (34 → 64). The US Title 42 entry suspension on 18 May marks the first deployment of this authority since COVID-19 and signals maximal alarm. Dr. Peter Stafford, a US physician working in Bunia since 2023, was confirmed positive on 18 May and medevaced to Germany with six high-risk-contact Americans. The same day the CDC confirmed an independent second American case in Uganda. Critical gaps remain: no approved Bundibugyo-specific therapeutics or vaccines, weak contact-tracing in insecurity-affected zones, and four healthcare worker deaths in 72 hours at Mongbwalu General Referral Hospital signal catastrophic IPC failures. WHO and Africa CDC are convening R&D Blueprint expert review of candidate monoclonal antibodies, remdesivir, and BVD vaccine candidates for emergency-use clinical trials.

FOR POLICYMAKERS

- Activate cross-border surveillance and exit screening between DRC, Uganda, Rwanda, South Sudan, CAR and Kenya; brief national EOCs on BVD-specific clinical case definitions and rapid PCR panels that distinguish BVD from Zaire ebolavirus.
- Establish emergency-use clinical-trial protocols in DRC and Uganda for investigational BVD monoclonal antibodies, remdesivir (IV and oral), and pre-clinical BVD vaccine candidates; existing Zaire-targeted countermeasures are not assumed cross-protective.
- Scale isolation and ETC capacity in Bunia, Mongbwalu and Kampala; pre-position PPE and surge medevac air-ambulance contracts; brief receiving hospitals (Hamburg, London Royal Free, NIH-Bethesda, others) on BVD-specific case management and staff protections.

ACUTE EVENT · CONTACT-TRACING PHASE

Andes hantavirus

Orthohantavirus andesense (ANDV) — MV Hondius cruise-ship cluster

MV Hondius docked at Rotterdam 18 May 2026 · contacts across 19 countries

10

CASES (8 CONFIRMED, 2 PROBABLE)

3

DEATHS

19

COUNTRIES WITH POTENTIAL CONTACTS

Severity

8.3

Novelty

6.0

Attention

6.5

Policy

7.0

Pandemic

2.5

Composite: 6.1 / 10

WHY IT MATTERS

The MV Hondius docked at Rotterdam on 18 May 2026. The Dutch RIVM reported no symptomatic disembarking; the remaining 25 crew and 2 medical staff entered quarantine and are undergoing clinical monitoring. Andes virus is the only hantavirus with documented human-to-human transmission, and the incubation window extends to ~6 weeks. Repatriated passengers and crew are being followed in 19 home countries. WHO still assesses global public-health risk as low, but operational burden on border health services, receiving countries' contact-tracing systems, and international clinical-alert pathways is high. No new symptomatic cases have been reported among disembarking personnel or early contacts as of 24 May.

FOR POLICYMAKERS

- Sustain cross-border contact tracing for repatriated passengers and crew across 19 countries; incubation window extends up to ~6 weeks; median onset 14–17 days.
- Support the Dutch RIVM disembarkation and disinfection protocol; provide secondary clinical-alert pathways for any contact developing fever, cough or respiratory symptoms.
- Codify cruise-line protocols: rodent-exclusion, onboard isolation, cabin decontamination, passenger-screening for exposure history on embarkation.

SMOLDERING · PHEIC

Mpox (clade Ib)

Monkeypox virus, clade Ib

DRC, Madagascar, Uganda, Burundi, Kenya · sustained clade I transmission across ≥10 EU/EEA countries

80

CLADE I CASES · EU/EEA · MARCH 2026 (10 COUNTRIES)

247

CLADE I CASES · EU/EEA · JAN–MAR CUMULATIVE

77%

UNVACCINATED OR PARTIALLY VACCINATED AMONG CLADE I CASES

Severity

5.3

Novelty

3.8

Attention

4.3

Policy

6.8

Pandemic

4.2

Composite: 4.9 / 10

WHY IT MATTERS

ECDC's monthly mpox surveillance shows sustained clade I transmission in Europe, with 80 clade I cases reported across 10 EU/EEA countries in March 2026 (Spain leading with 23 cases), 78 in February and 89 in January — demonstrating persistent endemic circulation at moderate levels. Just 23% of reported clade I cases had completed a two-dose JYNNEOS / MVA-BN course, indicating persistent vaccination gaps despite two years of availability. Risk to MSM remains moderate; risk to general population remains low. The Bundibugyo ebolavirus outbreak and hantavirus cruise-ship cluster add operational burden on national health authorities in DRC and cross-border zones. A novel clade Ib/Ib recombinant remains flagged (WHO DON 2026-DON595) but not detected in recent surveillance.

FOR POLICYMAKERS

- Sustain targeted JYNNEOS / MVA-BN vaccine availability for at-risk MSM groups; address coverage gap — 77% of reported clade I cases were unvaccinated or partially vaccinated.
- Maintain non-stigmatizing messaging through community organizations and sexual-health clinics; avoid travel restrictions for clade Ib.
- Fund continued genomic surveillance — clade I/Ib recombinants are emerging; ECDC's new monthly reporting will track variants in real time.